



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : شادى سعد محمود محمد
المحافظة : القاهرة الحي / المدينة : المطرية
رقم البطاقة : 28707100100136 رقم التليفون : 01100822279
العنوان : القاهرة
العمر (تاريخ الميلاد) : 10-07-1987 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 24-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان الرئة
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان الرئة
خطة العلاج : أبحاث أورام أشعه مقطعيه علي المخ بالصبغه [10851] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: ----- التوقيع: -----
الاسم: د/ندى حسن الوظيفة: ----- التوقيع: -----
الاسم: د/نرمين رمضان الوظيفة: ----- التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: شادى سعد محمود محمد National ID: 28707100100136	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

A patient of lung cancer for Contrast-enhanced CT of the brain
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

السيد أ.د / مدير عام المجالس الطبية

تحية طيبة وبعد

كشف حساب

علاج على نفقة الدولة

بيان بنفقات المريض / **لشاني لاسر محمود**

عن الفترة من ٢٦ / ٣ / ٢٠٢٦ الى ١٧ / ٥ / ٢٠٢٦

البيان	وصيدل القوار	قيمة المنصرف
الرصيد	١٨١٨٣	
اقامة	0	
كشف	0	
اشراف طبي	0	
اشعه (رنين - مقطعية - اخرى)	0	
تحاليل طبية	0	
مستلزمات	0	
علاج متنوعه (بنك دم / علاج)	0	
الادويه	0	
المتبقي		٣٨٤



المراجع
الطبيب

الموظف المختص
رنا

جمهورية مصر العربية
بطاقة تحقيق الشخصية

شهادى
مبسط محمود محمد ابو الذهب
٤٥
السيدة زينب - القاهرة

١٩٨٧/٠٧/٢١ ٢٨٧٠٧١٠٠١٠٠١٣٦
LR8998169

٢٠٢١/٠٤ ٢٨٧٠٧١٠٠١٠١٣٦

حاصل على بكالوريوس في نظم
المعلومات الادارية
مستلم
منزوح
ذكر

البطاقة مارية حتى ٢٠٢٣/٠٤/١٠ ١

٢٠٢١/٠٤ ٢٨٧٠٧١٠٠١٠١٣٦

Patient's Name: Shadi Saad Mahmoud
Date: 2026-06-08
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

A known case of left lung mass for metastatic workup

I- CHEST:

- The left apical lobe showed soft tissue mass lesion, speculated borders and contains tiny calcific foci. It is seen measuring about 4.1x3.5 cm in its axial dimensions.
- Bilateral upper para-septal emphysematous bullae are noted, the largest is seen on right side measures about 7.1x7.4 cm
- Bilateral basal atelectatic bands are noted
- Mild right sided pleural effusion.
- No hilar or mediastinal masses or lymph node enlargement.
- No left pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.
- Elevated left copula of diaphragm

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- The examined skeleton No worrisome lytic or sclerotic lesions.

Patient's Name: Shadi Saad Mahmoud
Date: 2026-06-08
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Opinion:

- Left apical speculated mass lesion likely neoplastic for histopathological correlation.
- Bilateral upper lobar para-septal emphysematous bullae.
- Mild right sided pleural effusion.
- The rest of the examination is unremarkable

Dr.Nourhan Ramzy

Dr.Nourhan Ramzy

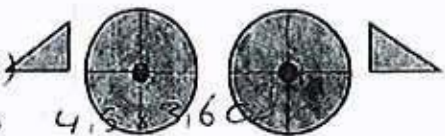
MDT FORM

1/2 / 2026

G

Name	منا ديسر كوند	MRN	3466
Age	39 y.o.	ID	695
Menopausal status		Phone	38988
NRS SCORE		Marital status	CAV.VI.I.I.I.I
BMI		Family history	
CO morbidity		REASON OF MDT	Aden. Carcin.

Clinical Data

BX:	Lung Adenocarcinoma			R	L
ER:	PR:	HER 2:	Ki67:		
Sonomammography: CT Chest (12-2-2024)					
Shungy: - Lt upper Soft tissue mass 4.5 x 3.6 cm - Bilateral pulmonary nodules					
Mets Workup: - Enlarged mediastinal LN 1.7 cm - pericardial effusion				TNM	
Pathology of lung mass: Lung Aden. Carcin.					

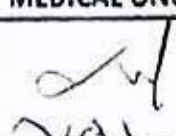
Recommendations:

TTF1 : +ve
 Napsin A : focally positive
 pt Admit to IC & Respiratory distress
 ALK & EGFR → R
 PET Scan →

MDT OUTCOME & PLAN

Dr. Alimta / Carboplatin & Re - 1.1.25
 for Results of ALK & EGFR

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
					

Name : Shadi Saad Mahmoud Mohamed Abo Al Dahab Date : 27-01-2026 09:41

Branch : Nasrcity

Medical No. : 300336

Ref.Physician Prof : Goustav Roussey Egypt Ins.Company : التأمين الصحي

Whole body (FDG) PET/CT Study:

Clinical history:

- 38-year-old male patient with history of left lung mass.
- PET/CT is requested for status evaluation.

Technique:

- o For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROI.s and the normal threshold is < 3.0

PET/CT findings:

Head and neck:

- Bilateral symmetrical tracer distribution is seen within the brain parenchyma with no abnormal metabolic activity or definite SOLs could be detected.
- Normal appearance of soft tissues and glandular structures of head & neck.
- No pathologically enlarged or tracer-avid cervical or supraclavicular LN.s.

Chest:

- Metabolically active left upper lobe consolidative mass lesion is noted measuring 4.1x3.7 cm with SUV max 6.5.
- Metabolically active pre tracheal, left internal mammary and aorto-pulmonary nodal lesions with SUV max 6.1.

Name : Shadi Saad Mahmoud Mohamed Abo Al Dahab Date : 27-01-2026 09:41

Al Dahab

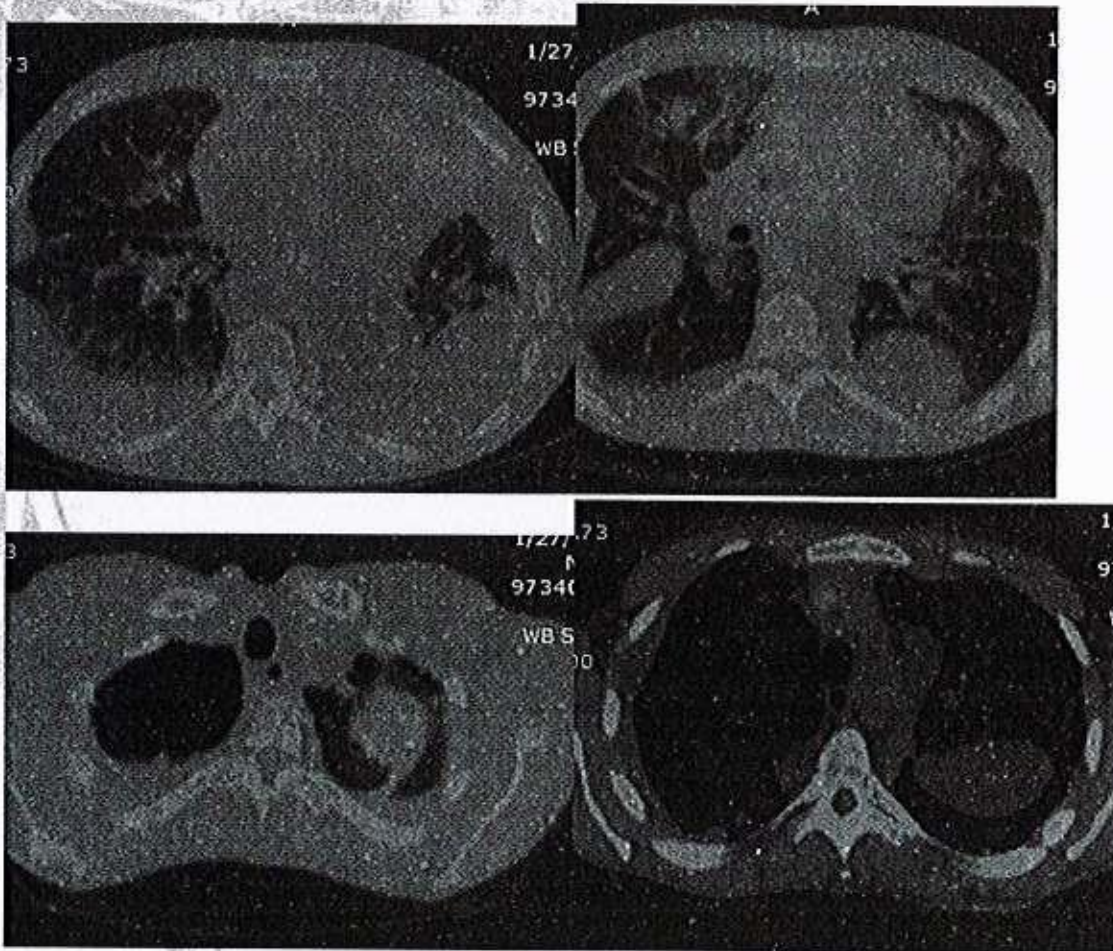
Branch : Nasrcity

Medical No. : 300336

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : التأمين الصحي

- Low grade metabolically active bilateral pleural nodular thickening associated with bilateral pneumonic and nodular opacities with SUV max 3.5.
- No current significant metabolically active lung nodules or parenchymal infiltrates.
- No enlarged or tracer-avid axillary LN.s.



Abdomen and Pelvis:

Name : Shadi Saad Mahmoud Mohamed Abo Date : 27-01-2026 09:41

Al Dahab

Branch : Nasrcity

Medical No. : 300336

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : التأمين الصحي

- Average-sized liver showing homogenous CT-texture and tracer uptake pattern with no definite focal lesions (on non-contrast CT basis).
- Unremarkable spleen, kidneys, adrenals, pancreas and pelvic organs.
- No pathologically enlarged or tracer-avid abdomino-pelvic LN.s.
- No evidence of ascites, free or encysted abdomino-pelvic fluid collections.

Musculoskeletal & BM:

- No any pathological sclerotic/lytic bony lesions or abnormal tracer-avid foci could be detected over the surveyed skeleton.

Impression:**Whole-body (FDG) PET/CT study is showing:**

- Metabolically active left upper lobe consolidative mass lesion associated with bilateral nodular opacities , pleural nodular thickening and regional nodal lesions for histopathological correlation.
- The rest of the surveyed body show no scintigraphic evidence of any other significant metabolically active lesions.

Best regards,

Dr. Ahmed dowydar, MSc.

Post Op to : شادي سعد محمود
Patient Id : Male 71Y
VisitId : Out Patient
Collection Date : 3466-2025

Patient Id : 4133
VisitId : 7323
Collection Date : 16/12/2025 12:14:49
Reporting Date : 16/12/2025 12:19:19

COMPLETE BLOOD PICTURE

Test	Result	Unit	Reference Range
Hemoglobin (Hb)	14.7	g/dL	13 - 17
Red Cell Count (RBC)	4.74	$10^6/\text{cmm}$	4.5 - 5.5
Hematocrit (HCT)	45.9	%	40 - 50
MCV	97.6	fl	76 - 95
MCH	31.0	pg	26 - 32
MCHC	33.5	g/dL	32 - 36
RDW	12.40	%	11.5 - 14.5
Platelet Count	392	$10^3/\text{cmm}$	150 - 450
WBC	H 12.97	$10^3/\text{cmm}$	4 - 10

Differential WBC Count

Relative

Absolute

	Relative (%)	R.Range	Absolute ($\times 10^3/\text{ul}$)	R.Range
Neutrophils	69.70	40 - 70	H 9.04	2 - 7
Lymphocytes	23.70	20 - 45	H 3.07	1 - 3
Monocytes	5.00	2 - 8	0.65	Up To 1.0
Eosinophils	1.40	1 - 6	0.18	Up To 500
Basophils	0.20	0 - 1	0.03	Up To 100

Signature
Dr. Radwa Mohammed

ECHO REPORT

NAME : شادي عاصم DATE: 28 / 12 / 202
ID NUMBER : ٣٤٦٦ / ٢٠٢٠ SEX : ♂ AGE: 39

EF	60%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.7	3.5-5.8cm	AO.R	2.7	3-3.7cm
LVEDS	2.5	2-4cm	LA	3.5	2-4cm
FS	28%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal
LV Wall Thickness Normal
LV Diastolic Function GI
SWMA No Rumt
LA and Ao Root Dimensions Normal
Mitral Valve
Aortic Valve
Tricuspid Valve Mild TR
Pulmonary Valve
Right Side and Systolic Function
Intracardiac Masses and Effusion

+ Large pericardial effusion in
multiple septations & Anterior
DR.
mostly accumulated anteriorly 3.5 cm.
No Tamponading Signs.

28/12/2020

Patient Name : شادي سعد محمود محمد
Patient Info : Male 71Y
Department : Out Patient
Profile NO. : 3466-2025

Patient Id : 4133
VisitId : 7323
Collection Date : 16/12/2025 12:14:45
Reporting Date : 16/12/2025 12:36:50

Coagulation Profile Report

Test	Result	Unit	Reference Range
Prothrombin Time (PT)	13.1	Sec	11 - 14
Partial Thromboplastin Time (PTT)	11.2	Sec	-
Fibrinogen	74.1	%	70 - 100
D-Dimer	1.17		1 - 1.3

Signature
Dr. Radwa Mohammed

CAIRO UNIVERSITY
FACULTY OF MEDICINE
PATHOLOGY DEPARTMENT

Pathology No: 10693

Name شادي سيد محمود محمد

Age: 39 Sex: male

Clinical diagnosis: Left upper lobe mass

Hospital_no: 287071001

Nature of specimen Biopsies

Department: 24

Date: 01/01/2026

PATHOLOGY REPORT

GROSS:

Four specimens:

1-Intra-pericardial tissue: Two irregular brown tissue pieces, collectively; 2.5x2.5cm, sliced & totally submitted.

2-Pericardial biopsy: A flattened brown tissue piece; 4x3cm, maximal wall thickness; 0.5cm.

3-Pericardial biopsy: A flattened tissue piece; 2x1.3cm. Maximal wall thickness; 0.3cm. Multisected & totally submitted.

4-Left upper lobe mass: Multiple irregular grey white and brown tissue pieces, collectively; 2.5x2.5cm, sliced & totally submitted.

MICROSCOPIC:

Examination of specimen (1) received revealed fibrin entangling fibrotic tissue pieces showing infiltrating neoplasia growth formed of clusters & acini, lined by epithelioid cells showing mild to moderate pleomorphism, nuclear hyperchromasia with clear to eosinophilic cytoplasm.

Examination of specimen (2) received revealed fibrotic fatty tissue pieces infiltrated by tumor tissue with similar morphology in specimen 1.

Examination of specimen (3) received revealed fibrofatty pieces entangling very few clusters of cells with hyperchromatic pleomorphic nuclei in vascular spaces.

Examination of specimen (4) received revealed lung tissue showing extensively necrotizing malignant neoplasm formed distorted acini, papillae & micropapillae, lined by moderately pleomorphic epithelioid cells with hyperchromatic nuclei.

CAIRO UNIVERSITY
FACULTY OF MEDICINE
PATHOLOGY DEPARTMENT

Pathology No: 10693

Name شادي سعد محمود محمد

Age: 39 Sex: male

Clinical diagnosis:

Hospital_no:

Nature of specimen

Department: 24

Date: 10/01/2026

PATHOLOGY REPORT

DIAGNOSIS:

1- Intra-pericardial tissue; biopsy:

Malignant epithelioid neoplasm, with focal glandular differentiation.

2- Pericardial biopsy:

Malignant epithelioid neoplasia with focal glandular differentiation.

3- Pericardial biopsy:

Few lymphovascular tumor emboli.

4- Left upper lobe mass, biopsy:

Poorly differentiated adenocarcinoma

Case for IPT on both lung & pericardial tumors.

N.B pericardial tumor (block 1-3), lung tumor block 5.

PROF. DR. RASHA KHAIRY
DR. NEHAL EL-LITHEY

د. رشا خايري
د. نهال الليثي

CYTOLOGY LABORATORY



Kass Al Ainy Hospital, Faculty of Medicine
Cairo University
Mandiat, Cairo, Egypt

PATHOLOGY DEPARTMENT

Dr. S. ELSHEIKH

Professor of
pathology
Head of Cytology

CYTOLOGY UNIT

Dr. S. MAHFOUZ

Prof. of Pathology
Senior cytologist

Dr. G. ASSAAD

Prof. of Pathology
Senior cytologist

Dr. M. AYDUB

Prof. of Pathology
Senior cytologist

Dr. A. HAREEDY

Lecturer Pathol
Senior cytologist

Dr. Y. FATHY

Ass. Prof. Pathol
Senior cytologist

Dr. S. IBRAHIM

Lecturer Pathol
Senior cytologist

Name:

محمد عبد الجبار

Age:

30

Sex:

M

Serial Number:

1000

Date:

24/11/2025

Hospital Num

0

Arabic Diagnosis:

10:

History Clinical Examination:

Pericardial effusion

Procedure: 20 cc hemorrhagic fluid.

Microscopic Examination:

Cytological examination revealed hemorrhage background entangling group of mesothelial cells with irritate forms and 30 balls of highly atypical cellular proliferation.

Diagnosis:

**PERICARDIAL EFFUSION, CYTOLOGY: SUSPICIOUS FOR
MALIGNANCY.**

**THE INTERNATIONAL SYSTEM FOR REPORTING SEROUS FLUID
CYTOPATHOLOGY CATEGORY IV.**

Revised by: MR

Signature:

PROF. DR. YASMINE FATHY

DR. MAHA EMAD EL DIN

DR. ESRAA ESMAT

AIRO UNIVERSITY
FACULTY OF MEDICINE
PATHOLOGY DEPARTMENT

Pathology No: 10693

Age: 39 Sex: male

Name: شادي سعد محمود محمد

Clinical diagnosis: Left upper lobe mass

Hospital_no: 287071001

Nature of specimen: Biopsies

Department: 24

Date: 01/01/2026

PATHOLOGY REPORT

GROSS:

Four specimens:

1-Intra-pericardial tissue: Two irregular brown tissue pieces, collectively; 2.5x2.5cm, sliced & totally submitted.

2-Pericardial biopsy: A flattened brown tissue piece; 4x3cm, maximal wall thickness; 0.5cm.

3-Pericardial biopsy: A flattened tissue piece; 2x1.3cm. Maximal wall thickness; 0.3cm. Multisected & totally submitted.

4-Left upper lobe mass: Multiple irregular grey white and brown tissue pieces, collectively; 2.5x2.5cm, sliced & totally submitted.

MICROSCOPIC:

Examination of specimen (1) received revealed fibrin entangling fibrotic tissue pieces showing infiltrating neoplasia growth formed of clusters & acini, lined by epithelioid cells showing mild to moderate pleomorphism, nuclear hyperchromasia with clear to eosinophilic cytoplasm.

Examination of specimen (2) received revealed fibrotic fatty tissue pieces infiltrated by tumor tissue with similar morphology in specimen 1.

Examination of specimen (3) received revealed fibrofatty pieces entangling very few clusters of cells with hyperchromatic pleomorphic nuclei in vascular spaces.

Examination of specimen (4) received revealed lung tissue showing extensively necrotizing malignant neoplasm formed distorted acini, papillae & micropapillae, lined by moderately pleomorphic epithelioid cells with hyperchromatic nuclei.